

HOW TO OPEN A NON-MEDICAL HOME CARE AGENCY in Arizona

No state license required to open — a private-pay & AHCCCS/ALTCS business.

A complete, plain-language, step-by-step startup guide — written so you can actually DO each step.

Figures current for 2026 · Every contact, rate & fee should be re-verified before you rely on it.

The Arizona model — read this first

Arizona is one of the easiest states in the country to open a non-medical home care agency, for one big reason: the state does NOT require a license to provide personal care, attendant care, homemaker, or companion services in people's homes. Arizona (through the Department of Health Services) licenses only skilled home HEALTH agencies — nursing and therapy under a physician's care. Non-medical “supportive services” are defined separately in statute and fall outside that licensure. So you can lawfully open and serve private-pay clients without a state home-care license. The catch: if you want to serve Medicaid (ALTCS) clients, you must register with AHCCCS and meet its strict caregiver-training and contractor rules. This guide walks the whole path — opening, then getting paid — broken down so a first-timer can follow it.

The Arizona distinctive — no state license for non-medical home care. A.R.S. § 36-151 defines a “home health agency” as one primarily providing skilled nursing/therapeutic services under a physician's care, and separately defines non-medical “supportive services” (homemaker, meals, transportation) as OUTSIDE that definition. Result: a personal-care/attendant-care/homemaker/companion agency can operate without an ADHS license. You'll still form a business, get insured, and (in most cities) get a local business/TPT license — but there's no state health-facility license to obtain for non-medical care. Confirm the line with counsel before you rely on it, and never provide skilled/medical care without the ADHS home-health license.

Medicaid (ALTCS) changes the rules. The moment you want to bill Arizona Medicaid, you must become an AHCCCS-registered provider (Provider Type 40, Attendant Care) and meet the Direct Care Worker training/testing and background standards (Part 5) and use EVV. So there are really two versions of this business: a light-touch PRIVATE-PAY agency you can start almost immediately, and an ALTCS provider with real compliance obligations. Most agencies start private-pay and add ALTCS. Part 8 covers both.

How to use this guide

Read it once start to finish, then use it as a checklist and a phone list. It's written for someone who has never opened an agency — where we name a thing, we break it down and tell you exactly where to go and what to ask. It is general information, not legal, tax, or clinical advice; verify each step with AHCCCS and qualified counsel (see Part 11) before you rely on it.

The journey at a glance

1	Decide this is your business	A non-medical home-care agency — recurring, hourly revenue (Part 1)
2	Form the business & insure	Entity, EIN, NPI, insurance & bonding, local license (Part 3)
3	Open for private pay	No state license needed — start serving private-pay clients (Part 4)
4	Set your caregiver standards	Fingerprint Clearance Cards, CPR, background checks (Part 5)
5	Decide on ALTCS	Whether/when to become an AHCCCS

		provider (Parts 5 & 8)
6	Set up operations	Policies, scheduling, and (for ALTCS) EVV (Part 6)
7	Register with AHCCCS	APEP Provider Type 40 + the DCW training standard (Part 8)
8	Contract with ALTCS plans	Mercy Care, Banner-UFC, UnitedHealthcare (Part 8)
9	Get your first clients	Discharge planners, AAAs, the huge AZ senior market (Part 10)
10	Who to call & what to ask	Every contact + the exact questions, in one place (Part 11)

Rule of thumb. Because there's no state license gating you, your fastest path to revenue is private pay. Form the business, insure it, set caregiver standards, and start. Add ALTCS when you're ready for its training/EVV/contracting requirements — that's where the volume is, but it takes months to set up.

PART 1

What This Business Is

In short: A non-medical home care agency — caregivers who help clients with daily living in their own homes. No ADHS license required; paid by private pay and (if you enroll) Arizona Medicaid/ALTCS.

Arizona law distinguishes “home health services” (skilled, medical, physician-directed — licensed by ADHS) from non-medical “supportive services” (help maintaining a person at home: homemaker, personal care, attendant care, companionship). Your business provides the supportive, non-medical services: help with the activities of daily living (bathing, dressing, grooming, toileting, transferring, eating, mobility), plus homemaker tasks (meals, housekeeping, laundry, errands) and companionship. Because these aren't “home health services,” they don't require an ADHS home-health license.

Non-medical vs. skilled home health

	Non-medical (this guide)	Skilled home health
What it is	Personal/attendant care, homemaker, companion	Nursing, therapy under a physician's care
State license	NONE required	ADHS home-health agency license (A.A.C. R9-10)
Who delivers it	Direct care workers / caregivers	RNs, LPNs, PT/OT/ST
Medications	Reminders/assistance only	Nurses administer/manage medications

The Medicaid term: “Direct Care Worker.” When you serve ALTCS clients, your caregivers are “Direct Care Workers” (DCWs) and must meet AHCCCS's training/testing standard (Part 5). For private-pay-only work, that specific standard isn't state-mandated — but it's smart to train to it anyway.

Key terms. ADHS = AZ Dept. of Health Services (licenses skilled home health, not you). AHCCCS = Arizona's Medicaid agency. ALTCS = Arizona Long Term Care System (Medicaid long-term care). APEP = AHCCCS Provider Enrollment Portal. DCW = Direct Care Worker. FCC = Fingerprint Clearance Card. EVV = Electronic Visit Verification (Sandata). AWC/SDAC = Agency with Choice / Self-Directed Attendant Care. TPT = Transaction Privilege Tax (AZ's sales-tax/business registration). NPI = National Provider Identifier.

Why this business — and why now

Demand for in-home care is rising fast, and for reasons that aren't going to reverse: people overwhelmingly want to age in their own homes rather than move to a facility; the 65-and-over population grows every year; and hospitals now discharge patients “quicker and sicker,” which sends more families looking for help at home right after a hospital stay. Arizona has one of the fastest-growing 65-and-over populations in the country, anchored by huge retiree and snowbird communities — Sun City, Scottsdale, Green Valley, Mesa, and the greater Phoenix and Tucson metros — which means a deep, steady pool of families who want and can arrange in-home care. And it's a business you can start lean — there's no building to buy or fill, the revenue is recurring and hourly, and a good reputation compounds through referrals.

The three mistakes that sink new agencies — avoid these. New home-care owners rarely fail because of the care itself. They fail for three business reasons: (1) undercapitalization — they run out of cash covering weekly payroll before Medicaid or insurers reimburse (see the cash-flow note in Part 9); (2) weak caregiver recruiting and retention — they can't reliably staff the shifts they're offered, so referral sources quietly stop calling; and (3) sloppy compliance — a missing background check, training record, or (for Medicaid) EVV visit that triggers a payback or a survey finding. Get your cash runway, your staffing pipeline, and your paperwork right, and the rest follows.

PART 2

Who Regulates You (and What Each One Does)

In short: *For private pay, there's no state health regulator over you — just your city (business license) and general business law. For Medicaid, AHCCCS/ALTCS and its plans set the rules and pay you.*

No ADHS license for non-medical care

The Arizona Department of Health Services licenses skilled home health agencies, not non-medical home care. So for a private-pay non-medical agency, ADHS is not your regulator — a genuine Arizona advantage. (If you ever add skilled nursing/therapy, that's when ADHS licensure under A.A.C. R9-10, Article 12 kicks in.)

AHCCCS / ALTCS and the health plans (if you serve Medicaid)

AHCCCS is Arizona's Medicaid agency; ALTCS is its long-term-care program, run through managed care. To bill, you register as an AHCCCS provider (through APEP) and then contract with the ALTCS Elderly & Physical Disability plans that pay you. AHCCCS sets the Direct Care Worker training standard and requires EVV. ALTCS Member Services: 1-888-621-6880.

Your city & the Corporation Commission

You form your LLC at the Arizona Corporation Commission (ACC), and most Arizona cities require a local business license and TPT registration for your office. There's no statewide general business license.

Arizona DPS (Fingerprint Clearance Cards)

The Arizona Department of Public Safety issues the Fingerprint Clearance Card your caregivers need (Part 5) — required for ALTCS DCWs and a strong best practice for all caregivers.

PART 3

Form Your Business & Insure — Step by Step

In short: *This is where most first-timers get stuck. We break each piece all the way down — what it is, where to go, exactly what to do, and what to have in hand.*

3.1 Form your legal entity (an LLC)

An “entity” is the legal business — almost always a Limited Liability Company (LLC) — that holds your license or registration, your contracts, and your bank account, and that shields your personal assets if the business is ever sued. In Arizona you form the LLC through the Arizona Corporation Commission (ACC), filing Articles of Organization online — the filing fee is about \$50 (roughly \$85 expedited). You'll name a “registered agent” (a person or service with a physical in-state address who receives legal mail — this can be you) and adopt an Operating Agreement (the internal document that sets out who owns what and who can make decisions).

- 1. File your Articles of Organization.** File online at ecorp.azcc.gov (ACC Corporations Division, 602-542-3026) — the filing fee is about \$50, or roughly \$85 if you expedite.
- 2. Name a statutory agent.** Arizona calls the registered agent a “statutory agent” — a person or service with a physical Arizona address who receives legal mail. This can be you.
- 3. Handle publication (county-dependent).** New AZ LLCs must publish a notice of formation in an approved newspaper for three consecutive weeks — EXCEPT in Maricopa and Pima counties, where the ACC's online posting satisfies the requirement. Check your county before you pay for a notice.
- 4. Adopt an Operating Agreement — and enjoy no annual report.** Adopt an Operating Agreement (who owns what, who decides) for your bank and partners. A genuine Arizona advantage: LLCs file NO annual report, so there's no yearly state renewal fee to track.

Two Arizona quirks worth knowing. Publication is required outside Maricopa and Pima counties (those two counties are exempt because the ACC posts the notice for you), and there is NO annual report for Arizona LLCs. Separately, get your city business license and TPT (Transaction Privilege Tax) registration for your office — there's no statewide general business license.

3.2 Get your EIN — the free federal tax ID

An EIN (Employer Identification Number) is your business's federal tax number — think of it as a Social Security number for the company. It's free, takes about ten minutes, and you need it before you can open a bank account, run payroll, or enroll with any payer. Here's exactly how to get it:

- 1. Go to IRS.gov and open the EIN Assistant.** Search “Apply for an EIN online.” Apply directly on the IRS website — never pay a third-party site that charges for this free service.
- 2. Choose your entity and enter the responsible party.** Select “Limited Liability Company,” then give the responsible party's legal name and SSN or ITIN (usually you, the owner), plus your business name and address.
- 3. Finish in one sitting.** The online session times out after about 15 minutes of inactivity and can't be saved partway — have your details ready before you start.
- 4. Save your EIN and the CP-575 letter.** You get the EIN on the spot and can download the official CP-575 confirmation letter — save both; your bank and every payer will ask for them.

3.3 Get your NPI — the national provider ID

An NPI (National Provider Identifier) is a free, ten-digit ID number that health-care payers use to identify your organization. You'll use it to enroll with AHCCCS/ALTCS and bill for attendant, personal, and homemaker care. Your AGENCY needs a Type 2 (Organizational) NPI — which is different from the Type 1 (individual) NPI a single person would get.

- 1. Create an I&A account.** At nppes.cms.hhs.gov, set up an Identity & Access (I&A) account for whoever will manage the number — usually an owner.

- 2. Start a Type 2 (Organizational) application.** Log into NPPES and choose Type 2, not Type 1. Type 1 is for an individual person; your business is a Type 2 organization.
- 3. Enter your legal name, EIN, and address, and pick your taxonomy.** Choose the taxonomy that fits a home-care / personal-care agency. The “taxonomy” is the code that says what kind of provider you are — confirm the exact one your Medicaid program wants before you submit.
- 4. Submit — it's free and quick.** It usually processes in about 10 business days. Save the NPI; you'll use it on every enrollment and claim.

3.4 Open a business bank account

Keep the business's money completely separate from your personal money — it protects your liability shield and makes payroll, taxes, and payer deposits clean. Open the account once your entity and EIN are set up.

What to bring to the bank:

- Your EIN confirmation (CP-575) from the IRS
- Your filed Articles of Organization / Certificate of Formation and the entity ID number
- Your signed Operating Agreement
- A Beneficial Ownership form (the bank collects details on anyone owning 25%+ plus a control person)
- A government photo ID, SSN, date of birth, and home address for each owner/signer
- An opening deposit (often around \$100)

Where to go: a bank with lots of Arizona branches — Chase, Wells Fargo, or Bank of America, or a strong local option like Arizona Financial Credit Union or Desert Financial Credit Union. Home care is payroll-heavy — you pay caregivers weekly, but Medicaid and insurers pay you weeks later — so ask specifically about a business line of credit or invoice financing to bridge payroll before you scale.

3.5 Get your insurance — what you need, who sells it, and what to ask

A home-care agency sends caregivers into clients' homes, so you need coverage built for that risk. Arizona has no mandatory state surety bond for non-medical licensure, but payers and families still expect a dishonesty bond:

Coverage	What it protects against	Must-have?
General & Professional Liability	A slip-and-fall or property damage in a client's home, plus claims about the CARE (a caregiver's error or neglect)	Yes — foundational
Abuse & Molestation	Allegations of abuse by a caregiver — general liability EXCLUDES this	Yes — high exposure
Employee Dishonesty / Surety Bond	Theft from a client's home — families ask if caregivers are “bonded”	Yes — clients & payers expect it
Non-Owned & Hired Auto	Your caregivers driving their own cars to clients or with clients	Yes — critical for home care
Workers' Compensation	Caregiver injuries (lifting/transferring)	Required in Arizona for employees

No state bond, but carry a dishonesty bond anyway. Arizona does not require a surety bond to operate a non-medical agency — but an employee-dishonesty bond is inexpensive, and ALTCS plans, the VA, LTC insurers, and families routinely ask whether your caregivers are “bonded.” Line it up with your liability coverage.

Real places to get it

Buy through an independent agent or broker who writes home-care programs. Carriers and programs active in this niche include Philadelphia Insurance (PHLY), CNA, The Hartford, and Markel, plus home-care specialty programs from Caitlin Morgan and NIP Group / HomeCareUSA. To find a local Arizona agent, use [trustedchoice.com](https://www.trustedchoice.com) (filter for home care). Confirm the carrier writes non-medical home care in Arizona before you rely on a quote — payers (ALTCS plans, the VA, LTC insurers) will require proof of coverage.

What to ask the insurance agent

“Do you write non-medical home care in Arizona — which carriers, and what's the minimum premium?”

“Is abuse & molestation included or a sub-limit, and do you include non-owned & hired auto (my caregivers drive their own cars)?”

“Can you place an employee-dishonesty / surety bond so I can tell families my caregivers are bonded?”

“What coverage limits do the ALTCS plans, the VA, or long-term-care insurers require?”

“What will Arizona workers' comp cost given my caregiver payroll?”

PART 4

Open Your Agency — No State License Needed

In short: For private pay, there's no state home-care license to get — so “opening” means forming the business, insuring it, setting caregiver standards, and getting local permits. You can start serving clients right away.

4.1 What “opening” actually requires (private pay)

- ☐ Entity formed at the ACC (with publication if outside Maricopa/Pima)
- ☐ EIN obtained; city business license and TPT registration in place
- ☐ Insurance & bond bound (liability, abuse, non-owned auto, dishonesty, workers' comp)
- ☐ Caregiver standards set (Fingerprint Clearance Cards, CPR/first aid, background checks — Part 5)
- ☐ Your policies, service agreement, and forms ready

That's it — no state survey, no state health license, no waiting on an inspection. This is why Arizona is one of the fastest states to launch a non-medical agency.

4.2 When you DO need more

- **To bill Medicaid (ALTCS):** register as an AHCCCS provider (APEP Provider Type 40) and meet the DCW training and EVV rules (Parts 5, 8) — a real process, but it opens the largest market.
- **To provide skilled/medical care:** that requires an ADHS home-health agency license (A.A.C. R9-10) — a separate, heavier path. Don't drift into nursing tasks under a non-medical model.

Consider voluntary standards anyway. Because Arizona doesn't gate you, quality is your differentiator. Training caregivers to the AHCCCS “Principles of Caregiving” standard and pursuing accreditation (Part 9) signal quality to families, referral sources, and payers — and make the ALTCS transition easier later.

PART 5

Your Caregivers — Standards, Cards & Screening

In short: Private-pay caregiver rules are minimal by state law, but smart operators screen hard. For ALTCS, the Direct Care Worker training/testing and Fingerprint Clearance Card standards are mandatory.

5.1 The Fingerprint Clearance Card (all caregivers)

Arizona's Fingerprint Clearance Card (issued by AZ DPS under A.R.S. § 41-1758 / § 36-411) is the backbone of caregiver screening. DCWs serving ALTCS clients generally need a Level One card; for private pay it's a strong best practice. Have each caregiver apply through DPS (psp.azdps.gov), and keep the card on file.

5.2 ALTCS Direct Care Worker standards (mandatory for Medicaid)

If you serve ALTCS clients, your DCWs must meet AHCCCS's standard (ACOM Policy 429 / AMPM 1240-A):

- 1. Complete the standardized training.** The AHCCCS "Principles of Caregiving" curriculum — Level I (Fundamentals) plus Level II specialized modules — within 90 days of hire.
- 2. Pass the tests.** At least 80% on each knowledge test and 100% on the hands-on skills test. RNs/LPNs/CNAs and some ALF caregivers are exempt.
- 3. Keep current.** 6 hours of continuing education a year, plus current CPR/first aid with an in-person skills demonstration.
- 4. Screen and re-screen.** Criminal screening at hire and every 3 years; Adult Protective Services and DCS Central Registry checks annually.
- 5. Mind the 16-hour rule.** A DCW may provide no more than 16 hours of paid care in a 24-hour period.

5.3 Private-pay caregiver standards (set your own bar)

- **Screen everyone:** a background check, a Fingerprint Clearance Card, TB screening, and CPR/first aid — even where not state-mandated, these protect clients and win referrals.
- **Train to a standard:** use the AHCCCS Principles of Caregiving curriculum as your baseline so you're consistent and ALTCS-ready.
- **Employ, don't misclassify:** treat caregivers as W-2 employees; you handle payroll, workers' comp, and supervision.

5.4 Building your Direct Care Worker pipeline (the practical playbook)

The hardest ongoing job in Arizona home care isn't paperwork — it's keeping enough qualified caregivers on your roster to say yes when a discharge planner or an ALTCS case manager calls. Arizona doesn't gate hands-on caregivers behind a state certificate the way some states do, which makes it faster to hire — but the moment you serve ALTCS, every caregiver must clear a Fingerprint Clearance Card and meet the AHCCCS Direct Care Worker (DCW) training/testing standard. So your pipeline has two speeds: a fast private-pay track and a certified ALTCS track. Successful Arizona owners build both on purpose.

- 1. Recruit for the fast private-pay track first.** You can hire a caring, reliable applicant and put them to work on private-pay clients quickly — screen them (background check, Fingerprint Clearance Card, TB, CPR), orient them, and check them off before they work alone. Post continuously on Indeed, ZipRecruiter, and local Facebook caregiver groups, and pay a \$150–\$300 referral bonus to your best hires.
- 2. Certify your keepers to the DCW standard.** When a caregiver proves reliable, invest in taking them through the AHCCCS "Principles of Caregiving" curriculum — Level I (Fundamentals) plus the Level II specialized modules — and the 80%-knowledge / 100%-skills testing. This turns a private-pay caregiver into an ALTCS-billable DCW and deepens your bench for the volume market.

3. Front-load the Fingerprint Clearance Card. The Level One Fingerprint Clearance Card from AZ DPS (psp.azdps.gov) can take weeks. Start every promising caregiver's application on day one so an ALTCS assignment isn't held up waiting on a card — an idle authorized client is lost revenue.

4. Decide private-pay vs. ALTCS per caregiver, not per agency. You don't have to certify everyone. Keep some caregivers on private-pay-only work (faster to deploy, paid up front) and route your DCW-certified, card-holding caregivers to ALTCS hours. Matching the caregiver to the payer keeps your fastest hires earning while your certified bench serves Medicaid.

5. Consider partnering with a Principles of Caregiving trainer. Approved DCW training is offered by community colleges and private trainers across Arizona. A standing relationship with one gives you a steady flow of already-trained candidates and a place to send your own hires for certification — turning training from a bottleneck into a recruiting funnel.

The private-pay-to-ALTCS decision. Because Arizona lets you start caregivers on private pay immediately but requires the Fingerprint Clearance Card and the full DCW training/testing before those same caregivers can bill ALTCS, the smart play is to launch fast on private pay, identify your reliable keepers, and certify them into DCWs as you build toward ALTCS. Track each caregiver's card status and DCW completion in one place — an expired card or missing training record discovered in an AHCCCS audit can cost you paid hours.

5.5 Recruiting and keeping good caregivers — the real make-or-break

Every experienced agency owner will tell you the same thing: your business rises or falls on caregivers, not clients. Referral sources will send you all the clients you can handle — but only if you can reliably staff them. So treat recruiting and retention as your core operation, not an afterthought.

- **Recruit constantly:** post continuously (Indeed, local Facebook groups, word of mouth, and a caregiver-referral bonus), and make applying fast — the best caregivers take the first agency that calls them back and schedules an interview.
- **Hire for reliability and warmth:** specific skills can be trained; showing up on time and being kind cannot. Check references for dependability, not just experience.
- **Onboard properly:** a real orientation, a skills check-off before a caregiver works alone, and a written scope of what they may and may not do — this protects clients and builds the caregiver's confidence.
- **Retain deliberately:** consistent hours, quick and correct pay, being reachable when a caregiver has a problem, genuine recognition, and a small raise ladder. Turnover is the biggest hidden cost in home care — every caregiver who quits costs you recruiting, training, and often the client they served.

PART 6

Policies, Operations & (for ALTCS) EVV

In short: *Run a professional operation: written policies, a clear service agreement, scheduling and billing systems, and — if you bill Medicaid — Electronic Visit Verification.*

6.1 Your policies & service agreement

Even without a state survey, write real policies: client rights and confidentiality, intake and the service plan, caregiver conduct and scope, supervision, complaint handling, incident and abuse/neglect reporting, and emergency procedures. Your service agreement should state clearly what you do (non-medical) and don't (skilled/medical) provide, the schedule and rate, and how service ends.

6.2 EVV for ALTCS (before your first Medicaid visit)

AHCCCS requires Electronic Visit Verification for attendant care, personal care, homemaker, habilitation, and respite. Arizona uses an open EVV model with Sandata as the state system (you may use the free Sandata solution or an approved alternate vendor). A Medicaid visit must be captured in EVV to be paid. Private-pay work isn't subject to EVV.

6.3 Systems

Pick a home-care software platform for scheduling, caregiver/client records, visit tracking, invoicing, and payroll — and, if you'll do ALTCS, one that supports EVV.

6.4 Documentation & staying inspection-ready

Whatever your state requires, the agencies that pass surveys and payer audits cleanly all do the same thing: they keep complete, current files from day one instead of scrambling the week before an inspection. Build these habits now, because reconstructing records after the fact is nearly impossible:

- **A file for every caregiver:** the application, background checks, training and competency records, health/TB screening, and a signed acknowledgment of your policies — all dated and kept current.
- **A file for every client:** the assessment, the service/care plan, the signed service agreement, consents, ongoing visit notes, and any payer authorizations.
- **A visit record for every shift:** who worked, when they arrived and left, and what care was provided — and, for Medicaid, the matching EVV record.
- **An incident and complaint log:** every incident, injury, and complaint and exactly how you resolved it — inspectors look for a real, used quality-improvement process, not a binder that's never been opened.

PART 7

The Client's Journey — Intake to Discharge

In short: A client is referred (privately or by an ALTCS case manager), you assess and set up the service plan, a caregiver delivers the hours (with EVV for ALTCS), you supervise, and you review and adjust.

Stage 1 — Referral & inquiry

A referral comes from a family, a hospital discharge planner, a home-health/hospice partner, or an ALTCS case manager (Part 10). Take the basics — who needs care, what tasks, how many hours, where, when, and who pays (private, ALTCS, VA, or LTC insurance).

Stage 2 — Assessment & the service plan

Do an in-home assessment and build a service plan: the tasks, schedule, and hours, and any safety notes. For ALTCS clients, the plan's case manager sets the authorized service and hours; your plan aligns to it.

Stage 3 — Agreement & start of care

The client (or representative) signs a service agreement (services, schedule, rate/payer, rights, termination). Match a screened, card-holding, competent caregiver and start with a supervised introduction.

Stage 4 — Delivering the hours (with EVV for ALTCS)

The caregiver delivers care; ALTCS visits are captured in EVV. Keep visit notes and protect the client-caregiver match — consistency drives satisfaction and retention.

Stage 5 — Supervision & quality

Supervise per your policy; monitor performance and changing needs; run your quality program; and report any suspected abuse, neglect, or exploitation.

Stage 6 — Changes & reassessment

As needs change, update the service plan; for ALTCS clients, coordinate reassessments and new authorizations with the case manager. Keep authorizations current so delivered hours get paid.

Stage 7 — Discharge & transition

When care ends, follow your discharge policy, give notice, coordinate with the family and any plan, and document. A good discharge turns families into referral sources.

PART 8

How You Get Paid — Private Pay, ALTCS, VA & LTC Insurance

In short: Start on private pay and long-term-care insurance right away; add Arizona Medicaid (ALTCS) by registering with AHCCCS. Attendant/personal care is billed by unit/hour.

Your four revenue streams:

Source	What it is	Key fact
Private pay	Client/family pays out of pocket, hourly	Start immediately — no state license; ~\$35/hr in AZ (2024)
Medicaid (ALTCS)	Arizona Long Term Care System attendant/personal care	The big market; requires AHCCCS registration, DCW training & EVV
Long-term-care insurance	The client's LTC policy reimburses in-home care	Bill the family or insurer once benefit triggers are met
Veterans (VA)	VA Homemaker/HHA, Veteran-Directed Care, Aid & Attendance	Serve veterans via the Community Care Network (Region 4, TriWest)

8.1 Private pay — your fast-start market

With no state license gating you, private pay is your quickest revenue. Arizona has a very large 55+/retiree market (Sun City, Scottsdale, Green Valley). Bill hourly (often with a visit minimum); Arizona private-pay non-medical care runs roughly \$35/hour (2024 cost-of-care data — verify current). Long-term-care insurance is part of this stream: many policies reimburse in-home personal care once ADL/cognitive triggers are met — you bill the family or the insurer.

8.2 Medicaid — ALTCS (the volume market)

To serve Arizona Medicaid clients, register with AHCCCS and contract with the ALTCS plans. ALTCS covers Attendant Care (combined homemaking + personal care + supervision), Personal Care, and Homemaker services.

- 1. Register through APEP.** Enroll in the AHCCCS Provider Enrollment Portal as Provider Type 40 (Attendant Care); attest to the DCW training/recordkeeping rules and disclose your employees.
- 2. Meet the DCW standard (Part 5)** and set up Sandata EVV (Part 6).
- 3. Contract with the ALTCS E/PD plans.** As of 2026 the operating Elderly & Physical Disability contractors are Mercy Care, Banner–University Family Care, and UnitedHealthcare Community Plan (the plan lineup was in flux after a procurement challenge — verify the current contractors and your service area with AHCCCS).

There are also member-directed options — Agency with Choice (a co-employer model) and Self-Directed Attendant Care (the member is the employer) — that your agency can support.

8.3 The rate & billing

Attendant care, personal care, and homemaker are billed under HCPCS codes (commonly T1019/T1020 for personal care and S5125/S5130 for attendant/homemaker) on the AHCCCS capped fee schedule — pull the current codes and per-unit/hourly rates from AHCCCS rather than relying on a quoted number. Bill against verified EVV visits, and confirm the timely-filing window with each plan.

8.4 Veterans

- **VA Homemaker / Home Health Aide & Veteran-Directed Care:** the VA's in-home programs — contract through the VA Community Care Network. Arizona is in Region 4, administered by TriWest (Phoenix-based).
- **Aid & Attendance:** an increased VA pension (2025–2026: up to about \$2,424/month for a single veteran, \$1,558 for a surviving spouse — verify on VA.gov) that a veteran can apply toward your private-pay care.

PART 9

What It Costs & How Long It Takes

In short: Startup is low — no state license fee. The real investments are insurance, payroll float, and (for ALTCS) the training/EVV setup and accreditation.

9.1 Startup line items (typical — verify)

Item	What's in it	Ballpark
State license	None required for non-medical	\$0
Business setup	ACC LLC ~\$50 (+ publication if applicable), EIN free, NPI free, city license/TPT, legal	~\$50–\$200 + legal
Insurance & bond	Liability + abuse + non-owned auto + dishonesty bond + workers' comp	Get quotes — annual
Caregiver screening	Fingerprint Clearance Cards, CPR/first aid, background checks	Per caregiver
DCW training (for ALTCS)	Principles of Caregiving curriculum + testing	Per DCW
EVV & software	Sandata (free) + scheduling/billing/payroll platform	Monthly SaaS
Payroll float	You pay caregivers before ALTCS/insurers pay you	Working capital
Accreditation (optional)	ACHC / CHAP / Joint Commission	~\$7,000–\$14,500 / 3-yr cycle

The cash-flow trap to plan for. You pay caregivers weekly, but ALTCS and insurers pay weeks later. Private-pay clients smooth this out — line up a business line of credit before you scale ALTCS hours.

9.2 If you pursue accreditation

Arizona doesn't require accreditation for non-medical home care, but ACHC, CHAP, or The Joint Commission accreditation is favored by the VA/TriWest and some LTC insurers and referral partners, and signals quality. Expect roughly \$7,000–\$14,500 over a 3-year cycle. Most agencies add it in year two.

9.3 Timeline

Week 1–2	Form & insure	ACC LLC, EIN, city license/TPT, bind coverage & bond
Weeks 2–3	Set caregiver standards	Fingerprint cards, CPR, background checks, training baseline
Right away	Start private pay	No state license to wait on — serve private-pay clients
Months	Add ALTCS	APEP Provider Type 40 + DCW training + Sandata EVV + plan contracts
Ongoing	Referrals & census	Discharge planners, AAAs, the AZ senior market

9.4 The money math — a simple break-even

Home-care economics are simple once you see them laid out. On each private-pay hour, you bill the client a rate; out of that you pay the caregiver's wage plus payroll taxes, workers' comp, and your overhead. The gap is your gross margin — often roughly 30–40% of the bill rate for a well-run private-pay agency. A worked example, using round numbers you'll replace with your own:

- **Bill rate to the client:** say \$35/hour (set yours from your local market and what referral sources see).
- **Fully-loaded caregiver cost:** the wage (say \$15) plus about 15–20% for payroll taxes, workers' comp, and paid time — roughly \$18/hour all-in.
- **Gross margin:** about \$12/hour, before overhead — from which you cover insurance, software, office, marketing, and your own pay.

- **What this means for you:** you need enough billed hours each week to cover your fixed overhead before you earn a profit — which is why census (total authorized client hours) and caregiver utilization are the two numbers to watch, and why a few reliable, long-hours clients are worth more than many tiny visits.

PART 10

Getting Your First Clients

In short: Clients come from discharge planners, home-health/hospice partners, the Area Agencies on Aging, ALTCS case managers, care managers, and Arizona's large senior communities. Build these before you open.

10.1 The referral sources

- **Hospital discharge planners & case managers:** the highest-volume source for post-hospital home support — make same-day starts easy.
- **Home-health & hospice partners:** they refer the non-medical hours their benefit doesn't cover; build reciprocal partnerships.
- **Area Agencies on Aging:** Arizona has seven AAA regions (coordinated by AZ4A) covering all 15 counties — Region I (Maricopa), Pima Council on Aging, NACOG, WACOG, Pinal-Gila, SEAGO, and the Inter Tribal Council. They field families needing in-home help — get listed and known.
- **ALTCS case managers:** once contracted, plan case managers route member hours to in-network agencies.
- **Senior communities & care managers:** Arizona's large 55+/retiree communities and Aging Life Care professionals refer supplemental one-on-one care.
- **2-1-1 Arizona:** list your services with 2-1-1 (dial 211) — a free statewide referral channel.

10.2 What to ask each referral source

"How do you decide which home-care agency to refer a family to?"

"What do you need from me to be on your referral or preferred-provider list — license, insurance, bonding, Medicaid enrollment, availability?"

"Who exactly is the discharge planner, social worker, or case manager I should build a relationship with?"

"How fast do you need us to be able to start care after a referral — same day, 24 hours, 48 hours?"

"What makes an agency easy to work with, and what gets one dropped from your list?"

Speed and reliability win home-care referrals more than anything else: be the agency that answers the phone on the first call, can start care within 24–48 hours, communicates back to the referrer, and never no-shows a shift. One reliable start earns you the next ten referrals.

10.3 Your first-90-days referral plan

Referrals don't happen by accident — they come from a handful of relationships you build on purpose. Here's a simple plan for your first 90 days:

1. **Make a target list.** Write down the 10–15 hospital discharge planners, skilled home-health and hospice liaisons, senior-community directors, and case managers in your service area — these are the people who send families to agencies.
2. **Show up in person with a one-pager.** A clean one-page sheet: who you are, what you do, your service area, how fast you can start, your insurance/credentials, and your phone number. Leave it; then follow up a week later.
3. **Be absurdly responsive.** Answer every call, say yes to same-day starts whenever you can, and report back to the referrer after the first visit — that single habit earns the next referral.
4. **Ask for feedback and the next referral.** After a good start, ask the referrer what would make you easier to work with, and whether they have another family who needs help. Then do it again next week.

PART 11

Who to Call & Exactly What to Ask — Every Verify, in One Place

In short: Your master contact + verification hub. Everywhere the guide says “verify,” the who and the what-to-ask are collected here.

Opening, caregivers & the (no) license

Who	Contact	What to verify / ask
AZ Corporation Commission	602-542-3026 · ecorp.azcc.gov	Forming your LLC (~\$50); publication in your county; no annual report
Your city (business license/TPT)	your city's finance/tax dept.	The local business license and Transaction Privilege Tax registration
AZ DPS — Fingerprint Clearance Card	psp.azdps.gov	Caregiver clearance cards (Level One for ALTCS DCWs)
ADHS — Home Health Licensing (contrast)	azdhs.gov (Medical Facilities)	Only if you'll add SKILLED care — the home-health license
Legal counsel	a health-care/business attorney	Confirming your non-medical scope stays outside ADHS licensure

Business, insurance & billing IDs

Who	Contact	What to verify / ask
IRS	irs.gov (EIN Assistant)	Get your EIN (free)
NPES / CMS	npes.cms.hhs.gov	Your Type 2 NPI (for ALTCS/VA billing)
Insurance agent (home-care program)	trustedchoice.com ; carriers in Part 3.4	Liability + abuse + non-owned auto + dishonesty bond + workers' comp
Business bank	Chase/Wells/BofA; Arizona Financial / Desert Financial CU	Account fees; a business line of credit for payroll

Funding, EVV & referrals

Who	Contact	What to verify / ask
AHCCCS — provider enrollment (APEP)	azahcccs.gov/APEP	Enrolling as Provider Type 40; the DCW standard; disclosures
ALTCS E/PD plans	Mercy Care, Banner-UFC, UnitedHealthcare (Member Services 1-888-621-6880)	Which serve my area; contracting; the rate; authorizations; timely filing
AHCCCS — EVV / Sandata	azahcccs.gov (EVV)	EVV onboarding, the open-vendor model, and compliance
AHCCCS — fee schedule	azahcccs.gov (Rates & Billing)	The current attendant/personal-care codes and rates
VA Community Care / TriWest (Region 4)	vacn.triwest.com	The VA Homemaker/HHA program; joining the CCN

Referral & senior-services partners

Who	Contact	Role
Area Agencies on Aging (7 regions)	arizonaaging.org (AZ4A)	Local information & referral for older adults
2-1-1 Arizona	dial 211 · 211arizona.org	Free statewide referral line — list your services
Accreditor (optional) — ACHC/CHAP/TJC	achc.org · chapinc.org · jointcommission.org	Whether/when to accredit; payer requirements
Senior communities	Sun City, Scottsdale, Green Valley, etc.	Preferred-provider relationships for supplemental care

Common questions, answered

Do I have to take Medicaid?

No. Many successful agencies run entirely on private pay, long-term-care insurance, and veterans' benefits — it's often the smarter way to start, because you earn immediately without the enrollment, contracting, and EVV that Medicaid requires. You can add Medicaid later once you have cash flow and a team.

Can I run this from home at first?

Often yes for a small non-medical agency — the care happens in the client's home, so you may not need a storefront to start. Confirm your local zoning and your license/registration rules (some require a business address), and know you'll want real office space as you grow.

Should my caregivers be employees or independent contractors?

Almost always W-2 employees. Classifying caregivers as 1099 contractors to save on taxes is one of the most common — and most expensive — mistakes in home care; it invites wage-and-hour and tax liability, and most payers and insurers require employees. Treat them as employees and carry workers' comp.

How much cash do I really need to start?

Enough to cover several weeks — ideally a couple of months — of caregiver payroll before your first payer reimbursements arrive, plus your license/insurance/software setup. Under-capitalization is the number-one reason new agencies fail; line up a business line of credit before you scale.

How long until it's profitable?

Most agencies take several months to a year to build a steady client base and referral flow. Starting on private pay shortens the runway because you're paid quickly; Medicaid volume comes later and pays slower. Plan your finances for a ramp, not an overnight full census.

Do I need experience in health care?

It helps, but plenty of successful owners come from business, sales, or family-caregiving backgrounds and hire the clinical expertise they need (for example, a nurse where the state requires one). What matters more is being organized, responsive, good with people, and disciplined about compliance and cash.

What actually makes one agency win over another?

Reliability. Families and referral sources choose — and stay with — the agency that answers the phone, starts care fast, sends the same dependable caregiver, communicates, and never leaves a shift unstaffed. Get that right and referrals compound; get it wrong and no amount of marketing saves you.

You can do this

Opening an Arizona non-medical home care agency is about as low-barrier as it gets — no state license, no survey, and a huge senior market. Take it one rung at a time: form the business, insure it, set strong caregiver standards, and start on private pay; then decide whether to add ALTCS through AHCCCS for the volume. Verify each fee and contact as you go, and lean on the people in Part 11.

***Reminder.** Prepared for PolicyReady.org and grounded in A.R.S. § 36-151 / § 36-425.01 and current AHCCCS/ALTCS information as of 2026. This is general information, not legal, tax, or billing advice. Requirements, fees, rates, and contacts change — verify each with AHCCCS, your city, and qualified counsel before you rely on it. Confirm your non-medical scope stays outside ADHS home-health licensure, and the current ALTCS contractors and rates, before you rely on them.*